

Derbyshire Shared Care Pathology Guidelines

Lymphocytosis in Adults

Purpose of Guideline

Investigation and referral guidance for adult patients with lymphocytosis.

Definition

The normal range for the absolute lymphocyte count is age related.

In adults the normal range is $1.5 - 3.0 \times 10^9/L$

Lymphocytosis is defined as a lymphocyte count $> 5 \times 10^9/L$

What are the main causes of lymphocytosis?

Over 80% of patients have lymphocytosis as an incidental finding on a routine full blood count for unrelated symptoms or as part of health screening.

- Transient, reactive lymphocytosis is frequently seen in acute self-limiting viral infection, particularly infectious mononucleosis
- Smokers often have a mild but persistent lymphocytosis
- Chronic infections like tuberculosis, brucellosis, secondary syphilis
- Leukaemia and occasionally lymphoma. Chronic lymphocytosis is characteristic of chronic lymphocytic leukaemia, the incidence of which peaks between 60 and 80 years of age. In its early stages this condition is frequently asymptomatic and treatment is only required for significant progression, which is about 1% per year.

Please refer to the flowchart (see page 3).

Investigations in primary care for patients with lymphocyte count $>5 \times 10^9/L$:

- Request glandular fever screen if appropriate
- History of recent viral infection?
- Smoking history, chronic infection/inflammatory condition?
- Repeat FBC and blood film 6 weeks post viral infection and after quitting smoking. If lymphocytosis persists, blood film will be reviewed with a comment suggesting further action
- Unexplained lymphocyte count of $5 - 10 \times 10^9/L$ and otherwise well: monitor FBC 6 monthly

Criteria for non-urgent referral to Haematologists:

- Lymphocyte count of $>10 \times 10^9/L$, and otherwise well

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Authorised by Julia Forsyth

Criteria for urgent (2WW) referral to Haematologists:

- Lymphocytosis in association with:
 - Unexplained Hb <100 g/L or/and platelet <100 x 10⁹/L
 - B symptoms
 - weight loss >10% in previous 6 months
 - severe night sweats
 - unexplained fever of >38°C for >2 weeks
 - Lymphadenopathy
 - Unexplained hepatomegaly or splenomegaly or both

Contacts

Urgent queries:

RDH Consultant Haematologist
CRH Consultant Haematologist

Via RDH switchboard
Via Consultant Connect or on-call
Consultant through CRH switchboard
Via QHB Switchboard

QHB Consultant Haematologist

Non urgent queries:

RDH Consultant Haematologist
CRH Consultant Haematologist
QHB Consultant Haematologist

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Dr S Hebballi, Dr P Blackwell, Mrs H Seddon	August 2017	31 st August 2019
Dr R Faulkner, Dr K Lam, Dr E Welch, Dr M Ruparelia, Dr P Blackwell, Mrs H Seddon	June 2020	30 th June 2022
Dr J Tam, Dr K Lam, Dr E Welch, Ms H Seddon	Oct 2022	31 st Oct 2024
Dr J Tam, Dr K Lam, Dr E Welch, Dr F Al Kaisi, Dr P Blackwell, Ms S Knowles	Oct 2024	31 st Oct 2026

